

Our Ref: (T) mb/ss/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

CG8, G level, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Crohn's colitis

Iron deficiency anaemia

Medication: Infliximab x4 doses.

I reviewed [NAME] in clinic today following a call to the helpline. [NAME] has had four doses of Infliximab and was unsure [NAME] was developing skin problems and some hair loss. My initial impression was that [NAME] was developing a TNF-related psoriasis. On examination [NAME] does have some small patches of eczema behind her knees, small patch mid abdomen and some patches of potential psoriasis on her face behind her ears. [NAME] reports that these were worse initially after [NAME] has had her first Infliximab infusion and improve and settle down as [NAME] comes to her next dose. [NAME] is due her next Infliximab infusion on Thursday and this is probably why her symptoms have settled. [NAME] has obviously had significant side effects from a lot of the medications and I am therefore fairly reluctant to make any significant changes. I have informed [NAME] that whilst I do not expect her to tolerate significant side effects, I would like her to at least have a further dose of Infliximab and review things then. I have advised [NAME] to continue with mercaptopurine at the current dose of 50 milligrams daily and I have issued [NAME] with a two-month prescription for her to continue this. We are planning to meet up again following her next infusion to assess the side effects. Assuming all remains settled we can continue with Infliximab on her own with a possibility of reducing her mercaptopurine dose to see if this helps with any of the skin and hair loss. [NAME] has been reading up on other medications and is not keen on vedolizumab or methotrexate. For future information [NAME] is unable to self-inject and therefore either subcutaneous methotrexate or Humira or golimumab are not going to be an option. However I am hopeful that [NAME] can continue with the Infliximab infusions and particularly as it has had a quite dramatic improvement only in her bowels but also in her joints. [NAME] is happy with this plan and assuming all goes well will see her formally back in clinic in six months' time.

With kind regards

Yours sincerely

Sister [NAME]c (Hons)

Biologics CNS Independent Nurse [NAME] signature will go here DO NOT REMOVE